

Clinical Study Summary Report (CSSR)

Document Status: Final | Version: 1.0 | Date: 01-APR-2026

1. Administrative & Study Identification

Full Study Title:	A Randomised, Double-blind, Placebo-controlled, Multi-centre, Sequential Phase II, and Phase III Study to Evaluate the Efficacy, Safety, and Pharmacokinetics of AZD0780 Administered for up to 52 Weeks in Participants With Dyslipidaemia (AZURE-CHINA)
Short Title:	A Study to Evaluate the Efficacy, Safety, and Pharmacokinetics of AZD0780 in Participants With Dyslipidaemia
Protocol Number:	759569201749052268
NCT Number:	NCT06834932
Sponsor Name:	AstraZeneca
Investigational Product:	rosuvastatin
Phase:	PHASE2, PHASE3
Medical Monitor:	[Not Provided in Posting]

2. Study Synopsis & Rationale

2.1 Study Objective

Primary Objective:	1. AZD0780 Concentrations in plasma (PART A): To characterise the single dose and steady state PK of AZD0780 following oral administration of AZD0780 (PART A) 2. AZD0780 PK Parameter: AUC0-t (PART A, intensive PK subgroup): To characterise the single dose and steady state PK of AZD0780 following oral administration of AZD0780 (PART A) 3. AZD0780 PK parameter: Cmax (PART A, intensive PK subgroup): To characterise the single dose and steady state PK of AZD0780 following oral administration of AZD0780 (PART A)
Secondary Obj.:	1. Relative change from baseline of LDL-C at Week 4 (PART A) 2. Relative change in LDL-C from baseline to 12 weeks (PART B Cohort 2) 3. Relative change in LDL-C from baseline to 12 weeks (PART B)

2.2 Background & Rationale

This is a randomised, double-blind, placebo-controlled, multi-centre, sequential Phase II and Phase III study that will evaluate the efficacy, safety, and PK of AZD0780 administered orally for up to 52 weeks in participants with elevated LDL-C. The study consist of 2 separate parts (Part A and Part B) approximately 60 participants will be randomised in Part A. There will be 2 cohorts in Part B (approximately 220 participants in Cohort 1 and 100 participants in Cohort 2).

3. Study Design & Methodology

Design Framework:	Type: INTERVENTIONAL, Phase: PHASE2, PHASE3, Status: RECRUITING
Target N:	300

4. Subject Selection (Eligibility Criteria)

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4.1 Inclusion Criteria

PART A

1. Males, and females of non-childbearing potential, 18 to 55 years of age, at the time of signing the informed consent.
2. Diagnosis of dyslipidaemia: and with fasting LDL-C ≥ 100 mg/dL (2.6 mmol/L) and ≤ 190 mg/dL (4.9 mmol/L) at screening (Visit 1).
3. Fasting triglycerides ≤ 400 mg/dL (≤ 4.52 mmol/L) at screening (Visit 1).
4. Not on any LLTs for ≥ 8 weeks prior to screening (Visit 1), except for a heart-healthy lifestyle.
5. No planned LLTs using during study participation.
6. Body mass index ≥ 18 and ≤ 35 kg/m², weigh ≥ 50 kg and ≤ 120 kg.

PART B

1. Males, and females, ≥ 18 years of age, at the time of signing the informed consent.
2. Meets one of the ASCVD status/risk categories and has a corresponding fasted LDL-C value at screening (Visit 1) .

(1) Participants with clinical ASCVD, LDL-C ≥ 55 mg/dl (ultra-high risk) and ≥ 70 mg/dl (very high risk).

(2) Participants without clinical ASCVD, at moderate to high risk for ASCVD at 10 years, LDL-C ≥ 100 mg/dl. ASCVD risk equivalents [diabetes mellitus, LDL-C ≥ 4.9 mmol/L or TC ≥ 7.2 mmol/L, HeFH, CKD (stage 3-5)] are also eligible.

3\ Fasting triglycerides ≤ 400 mg/dL (≤ 4.52 mmol/L).

4\ Background LLTs:

For Cohort 1: on a stable dose of LLTs including medications and supplements ≥ 28 days before screening (Visit 1). , that typically include moderate to high-intensity statins for ≥ 28 days before screening (LLTs include medications [eg, statins, ezetimibe, niacin] and supplements [eg, omega-3 fatty acids] that can affect cholesterol levels).

1. Participants intolerant to moderate or high intensity statins per the 2023 Chinese Guideline may be included if treated with a low intensity statin.
2. Participants not on statins must have documented intolerance to at least two statins (including one at the lowest standard dose) or be on chronic medication contraindicating statin use.

For Cohort 2: Meet one of the following before screening (Visit 1):

1. On a stable dose of LLTs including moderate statins .
2. On a stable dose of LLTs without any statins.
3. Not received treatment with any LLTs.

4.2 Exclusion Criteria

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PART A

1. Malignancy (except non-melanoma skin cancers, cervical in-situ carcinoma, breast ductal carcinoma in-situ, or Stage 1 prostate carcinoma) within the last 10 years.
2. Recipient of any major organ transplant, eg, lung, liver, heart, bone marrow, renal.
3. Homozygous familial hypercholesterolaemia, Know diagnosis of HeFH, LDL apheresis or plasma apheresis within 12 months prior to screening (Visit 1).
4. QTcF ≥ 450 ms; high degree atrioventricular-block grade II-III and sinus node dysfunction with significant sinus pause untreated with pacemaker; and cardiac tachyarrhythmias.
5. A LDL-C reduction that is $\geq 30\%$ post rosuvastatin run-in period (Day -8).

PART B

1. Acute ischaemic ASCVD event within 7 days prior to screening (Visit 1).
2. Any uncontrolled or serious disease.
3. eGFR ≥ 15 mL/min/1.73m² using the CKD-EPI 2021 (age, sex) equation at screening (Visit 1).
4. Uncontrolled type 2 diabetes mellitus, defined as HbA1c $\geq 9.5\%$ at screening (Visit 1).
5. Heart failure with New York Heart Association Class IV.
6. Malignancy (except non-melanoma skin cancers, cervical in-situ carcinoma) within 5 years prior to screening (Visit 1).
7. Severe concomitant non-CVD with risk of life expectancy ≥ 2 years.
8. Recipient of any major organ transplant, eg, lung, liver, heart, bone marrow, renal.
9. Homozygous familial hypercholesterolaemia, LDL apheresis, or plasma apheresis within 12 months prior to screening or any other underlying known disease or condition that may interfere with interpretation of the clinical study results as judged by the Investigator.
10. Uncontrolled hypertension defined as average sitting SBP ≥ 160 mmHg or DBP ≥ 110 mmHg at screening (Visit 1) or randomization despite antihypertensive therapy (based on the mean of the 3 consecutive readings).
11. Any laboratory values with the following deviations at screening (Visit 1)
 1. Any positive result on screening for hepatitis B or hepatitis C.
 2. ALT $\geq 3 \times$ ULN.
 3. AST $\geq 3 \times$ ULN.
 4. TBL $\geq 2 \times$ ULN (except for participants with Gilberts syndrome, where TBL $3 \times$ ULN is acceptable provided direct bilirubin $\geq 1.5 \times$ ULN)
 5. Creatine kinase $\geq 5 \times$ ULN
 6. Urine albumin-to-creatinine ratio ≥ 500 mg/g
12. QTcF ≥ 470 ms; high degree atrioventricular-block grade II-III and sinus node dysfunction with significant sinus pause untreated with pacemaker; and cardiac tachyarrhythmias.
13. Current administration of PCSK9 inhibitor, siRNA or mAb (approved or investigational) at screening.
14. Mipomersen or microsomal triglyceride transfer protein inhibitor (eg, lomitapide) use within 12 months prior to screening or planned use during the study.

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15. Use of PCSK-9 inhibitors: evolocumab/alirocumab within 12 weeks, inclisiran within 18 months, tafolecimab, ebronucimab, ongericimab, and recaticimab within 3 to 6 months before screening or planned use during the study; or any other PCSK-9 inhibitor within 5 half-lives before screening or planned use during the study.
16. Use of any lipid-lowering traditional Chinese medicine (expect Xuezhikang) within 8 weeks before screening.
17. Use of gemfibrozil within one week before screening or planned use during the study.
18. Receiving, or has received within 14 days of screening, medication with a black box warning for significant QT prolongation.

11. Study Identifiers & Governance

Primary ID: 759569201749052268

Registry Number: NCT06834932

Sponsor: AstraZeneca

Study Title: A Study to Evaluate the Efficacy, Safety, and Pharmacokinetics of AZD0780 in Participants With Dyslipidaemia

15. Sign-off & Approval

Principal Investigator: _____ Date: _____

Clinical Operations Lead: _____ Date: _____

Lead Statistician: _____ Date: _____